

Supporting Assisted Suicide Legalization in Pennsylvania

Tsung-Yuan Lin

Department of Advertising and Public Relations,

Klein College of Media and Communication, Temple University

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Dr. Sandy Kyrish

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Lying still in bed, the patients stare at the ceiling with their unfocused eyes. They cannot take a shower by themselves, so someone cleans their body. They cannot hold the spoons, so someone feeds them. They cannot say a word, so someone speaks for them. They cannot do anything on their own. Their breathing becomes increasingly difficult, and the pain from organ failure overwhelms their body. To survive, they must take analgesics to alleviate the pain. Tubes and devices are all over their body, and they continue to suffer even though death is certain in a few months. In the meantime, it is the people who enjoy great health that decide whether the patients can end their unbearable condition. This issue seems unjust. As the lawmakers and politicians are pondering if the law would affect the rights of medical workers, the terminally ill patients suffer from their hesitation and constant worries. Unable to understand the pain the patients endure, the power to take initiative is held in their hands.

Pennsylvania should legalize assisted suicide and simultaneously ameliorate its long-term and palliative care. Realizing that the patients own their rights regarding living or dying, the lawmakers, politicians, and any related members must put concerted effort into releasing those who are suffering from unbearable disease. Furthermore, long-term and palliative care should also be taken care of since the main purpose is not to encourage people to resort to death. The patients should have options, and if fighting the illness is what they choose, the medical and care system should be there for them.

To understand the issue more comprehensively, some following information is necessary. First, the term "assisted suicide" should be referred to as MAID (Medical Aid in Dying). MAID describes "a trusted and time-tested medical practice that allows a terminally ill, mentally capable adult with a prognosis of six months or less to live to request from their doctor a

prescription for medication they can decide to self-ingest to die peacefully in their sleep" (Compassion & Choices, n.d.). The word "suicide" stigmatizes the policy and misleads people by showing patients' impulsiveness to kill themselves in a certain mental condition. Second, the Supreme Court ruled in *Washington v. Glucksberg* (1997) that states have the authority to regulate medical aid in dying. Therefore, Pennsylvania has the legal power to make this decision. Lastly, some states have already put it into practice and yielded remarkable results, especially Oregon's success. Overall, two positive and one opposing arguments will be discussed.

The demand for assisted dying has existed for over a hundred years, and it is time for the government to take the initiative. According to Sheldon (1906), people in the US started asking for assisted dying in the 18th century. However, there are only 11 states that allow MAID nowadays, which is still a small proportion of the whole country. Pennsylvania should emulate this practice, endeavoring to help the patients who are suffering. Two bills regarding MAID have been introduced to the Pennsylvania General Assembly: HB 1109 and SB 570. These bills have been in pending status since April 2025 (Compassion & Choices, n.d.). Additionally, for the past seven months, there has not been a single committee hearing, suggesting that the process is stagnant. Given the situation, the House and Senate should improve their responsiveness to the advocates. If the legislative bodies remain inactive, more people will have to endure the agony which healthy people cannot relate to. Recognizing that the right to live or die should be returned to the terminally ill patients, this paper will illustrate the problem with opposing perspectives.

Most arguments opposing MAID have a common limitation—they start from various perspectives but not the patients'. The barriers faced are usually concerns for the people involved in the system such as medics. A large portion of the public tends to defend the rights of healthy

people; however, those undergoing tremendous pain should not keep suffering. Greš, Staver, and Radovančević (2024) and Manu (2010) provide information concerning issues that happen in the medical field such as professional ethics, risk of abuse, and social consequences. They argue that assisting in ending life is a violation of the Hippocratic Oath, the oath of ethics historically taken by physicians. Moreover, assisted dying being illegally abused raises concerns for decision makers.

Oregon's 27-year experience completely contradicts the concerns. None out of 2,847 people has been proven to exploit the system according to Oregon Health Authority (2024), indicating the difficulties assumed by the scholars are just theoretical. The assisted dying policies can be comprehensively controlled and implemented. Medical workers can actively decide whether to participate. By legalizing MAID, the patients will regain control of their bodies and the Pennsylvania government will be able to strictly scrutinize the eligibility of patients. Most importantly, the rights of the patients will finally be prioritized and taken seriously. The benefits of legalizing MAID have been examined; proposals for implementation are made in the following paragraph.

Pennsylvania should pass HB 1109 and SB 570 to legalize MAID with Oregon's safeguards and simultaneously improve palliative care services. Foley and Hendin (2002) believe improving long-term and palliative care is the right decision. This surely will be conducive to saving more lives, but not everyone can endure such pain. Especially for terminally ill people, even by consuming analgesics, the pain isn't necessarily mitigated. As Sinha and Vashisht (2023) note, end-of-life pain management is often inadequate, showing that medication alone cannot guarantee relief. The frustration and affliction devour the patients both physically and mentally,

and their dignity will still be deprived. Consequently, assisted dying plays an indispensable role in the system with an attempt to provide everyone the right to live and die.

Granting them an option to die doesn't mean encouraging their death. Regarding Oregon's proven safeguard framework, it includes two-physician confirmation, mandatory waiting periods, mental health evaluations, and self-administration requirements (Oregon Health Authority, 2024). These measures have successfully prevented abuse for twenty-seven years while respecting patient autonomy. Considering Oregon's success, Pennsylvania is likely to yield similar results. Lastly, the medical workers should be assured the right to say no by the regulations. The doctors should not be forced to do things they are not comfortable with. After passing HB 1109 and SB 570, improving palliative care, adopting Oregon's safeguards, and securing the rights of medical workers, there is one last problem to address.

It is argued that MAID being legalized might indirectly force vulnerable groups to choose death, as they refuse to become burdens to their family and the society. Nevertheless, this concept assumes that terminally ill patients cannot make autonomous decisions. Oregon's safeguards make sure that the subjects of MAID are mentally evaluated. They can make their own choices since they are not mentally ill. Oregon's data indicate that of 4,274 people who received MAID prescriptions, only 2,847 (67%) actually used them (Oregon Health Authority, 2024). This proves people aren't being forced—having the choice brings comfort, but not everyone uses it. In the meantime, patients' most common reasons for taking the prescription were loss of autonomy (92%) and loss of dignity (64%), not economic pressure (Oregon Health Authority, 2024). The evidence shows our fears are not based on facts; legalizing MAID in Pennsylvania is beneficial and practical.

This research paper supports MAID legalization in Pennsylvania, examining the autonomy of patients, the biases in opposing arguments, actions Pennsylvania should take and the statistical evidence showing that patients are not being forced to use MAID. As more people come to realize the hardship that terminally ill patients endure, the autonomy and rights of these patients will be protected and upheld. Legalizing MAID is not only a legal adjustment but also a recognition of human dignity and self-determination. With proper safeguards and palliative resources, Pennsylvania will create a compassionate and safe framework that respects personal choices. Ultimately, respecting patient autonomy can create a more humane and comprehensive healthcare system.

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